

Armadale Kalamunda Group

Perinatal Loss Guideline

1. Purpose

The purpose of this guideline is to provide clinical staff with the information necessary to ensure the safe management of women experiencing perinatal loss: either fetal death in utero (FDIU), termination of pregnancy or neonatal death.

2. Applicability

This guideline applies to medical and nursing staff working within the Maternity Unit at Armadale Kalamunda Group.

Quick reference flowcharts are available in the appendices:

- [Appendix 6](#): Pregnancy Loss 14-20 Weeks Flowchart
- [Appendix 7](#): Perinatal Loss 20-28 Weeks Flowchart
- [Appendix 8](#): Perinatal Loss >28 Weeks Flowchart

3. Classification and Legal Identity

The [Births, Deaths and Marriages Registration Act 1998 WA](#) and its subsequent versions states, “It is compulsory to register the birth of a child whether born alive or stillborn”. In this Act, unless the contrary intention appears, definitions are as follows:

- “birth” means the expulsion or extraction of a child from its mother.
- “child” includes a still born child.
- “stillbirth” means the birth of a still born child.
- “Stillborn child” means a child
 - of at least 20 weeks’ gestation; or
 - if it cannot be reliably established whether the child’s period of gestation is more or less than 20 weeks, with a body mass of at least 400 grams at birth, that exhibits no sign of respiration or heartbeat, or other sign of life, immediately after birth.

The Perinatal Society of Australia and New Zealand (PSANZ) Perinatal Mortality Classification System is used in Australia and New Zealand to classify the causes of stillbirths and neonatal deaths. It includes the PSANZ Perinatal Death Classification (PSANZ-PDC) and PSANZ Neonatal Death Classification (PSANZ-NDC). The PSANZ-PDC system classifies all perinatal deaths by the single most key factor seen as the antecedent cause of death and can be viewed [here](#).

See Appendix 1 in this document for expanded information about reporting.

4. Causes and risk factors

It is vital to try determining the cause of stillbirth, however many cases remain unexplained (15-18% in WA). More than one condition may contribute to the stillbirth and conditions may be associated without directly causing the stillbirth. The proportion of stillbirths that are reported as “explained” increases when there is a systematic comprehensive approach to investigation.

Predisposing risk factors for perinatal death:**Maternal Social factors**

- Low socio-economic status
- Aboriginal and Torres Strait Islander
- Ethnicity: South Asian, African (including refugee or asylum seeker)
- Smoking
- Obesity
- Maternal age >35 years
- Nulliparity
- Substance Use

Pre-existing Medical factors

- Diabetes: Type 1 and Type 2
- Essential Hypertension
- Auto-immune disorders e.g., Systemic Lupus Erythematosus
- Other maternal e.g., Malaria, Sexually Transmitted Diseases
- Mental Health disorders

The most common causes are:**Antepartum**

- Congenital abnormality
- Perinatal infection
- Hypertensive disorders in pregnancy
- Antepartum haemorrhage
- Cholestasis of pregnancy
- Antiphospholipid syndrome
- Multiple pregnancy
- Post term
- Specific Perinatal Conditions: fetal maternal haemorrhage, uterine anomalies, autoimmune disease
- Fetal Growth Restriction
- Reduced fetal movements
- Placental dysfunction: e.g., abnormal maternal vascular perfusion

Intrapartum

- Hypoxic peripartum death with intrapartum complications
- Pre-viable preterm labour and birth

5. Diagnosis

Intra-uterine fetal death requires confirmation by a formal ultrasound examination that demonstrates an absence of fetal heart activity.

- The ultrasound should be performed by experienced staff (credentialed sonographer or obstetrician)
- A midwife or doctor as escort should be made available to support the woman while attending an ultrasound examination for confirmation of a suspected fetal death

6. Breaking sad news to the family

- Break sad news in a private and quiet room, the most experienced practitioners are required for these difficult conversations
- Ensure a support person is present for the woman
- Communication should be clear, empathetic and respectful, using language preferred by the patient and their family. Care should be taken with the terminology and use of certain phrases when discussing pregnancy loss with patients and their support people (e.g., the terms spontaneous abortion or failed pregnancy should not be used)
- Do not delay breaking the news once diagnosis is confirmed.
- Allow as much time as needed for parents to consider care options and make decisions
- Be aware that men and women may respond and grieve differently
- Staff are encouraged to express their sorrow for what has happened. Offering sympathy is not an admission of guilt or error
- Reassure parents that every attempt will be made to find a cause of death in a medical review if consented to

- Explain that stillbirths often remain unexplained even after a detailed review
- Avoid speculation regarding the cause of death until investigations are complete
- When appropriate, reassure the mother that the death was not due to anything she did or did not do
- Consider special circumstances (e.g. previous stillbirth or multiple pregnancy), cultural or religious needs
- Offer referral for counselling/support services (e.g. social worker, pastoral support, Red Nose “Hospital in the home” program)
- Discuss Creating memories

7. Considering birthing options

- Provide information on birth options appropriate to the clinical circumstances and service capabilities
- Vaginal birth is preferable to caesarean section with minimisation of maternal risk being the crucial factor
- There is usually no clinical need to expedite birth urgently and hasty intervention may not be in the best long-term interests of the parents. If clinically appropriate, the woman may wish to go home to return for induction later
- Consider referrals to the multidisciplinary team as appropriate (social work, pastoral care, anaesthetics)
- Consider method of induction relevant to gestation and clinical circumstances.
- Provide information to women and their families on how the baby may appear following birth. Parent’s fears are often worse than the reality. Be honest and use sensitive but unambiguous language

8. Investigations

These investigations should take place at the time of confirmed FDIU, prior to induction or delivery or following birth and in the postpartum period.

Refer to PSANZ Stillbirth Investigations Flowchart

At diagnosis of fetal death

Maternal History

- Take full maternal history

Fetal Examination

- USS to confirm FDIU
- Fetal abnormalities
- Growth & Amniotic fluid volume

Vaginal Swab

- HVS – Genital MC&S

Amniocentesis*

Maternal Blood Tests

- Full Blood Count
- Transfusion Medicine Sample Storage
- Foetal Maternal Haemorrhage

See [here](#) for Indicated Selective Investigations

Amniocentesis (for karyotype and infection screen) is a useful investigation; however, this is not offered at AHS

Following birth

Baby

- External Examination Checklist [here](#)
- Post-mortem examination
- Clinical Photographs
- Ear & nose surface MC&S swabs
- Newborn Screening Test for neonatal deaths **ONLY**

Please note: If patient consents to post-mortem, then it is not required to do clinical photos and ear & nose surface swabs as this is done at KEMH

Placenta and Cord

- Macroscopic examination of placenta and cord
- Placenta Surface MC&S Swabs if **NOT** consenting for postmortem – see Appendix 8
- Placental histopathology at AHS if **NOT** consenting to post-mortem

Please note: If a patient consents to post-mortem, then swabs and histopathology of placenta is done at KEMH

Indicated Selective Investigations as per PSANZ

Routine testing of all stillbirths for infection is no longer recommended. Targeted investigation should be undertaken if infection is suspected based on maternal history, autopsy and/or placental findings and/or a small for gestational age (SGA) baby. To assign infection as the cause of death, positive serology should be supported by autopsy and/or placental findings consistent with infection.

Personal or family history of Thrombosis - anticardiolipin, lupus anticoagulant, anti-B2 glycoprotein-1 antibodies

Suspected Cholestasis – bile acids & liver function tests

Large for gestational age - HbA1c

IUGR or SGA - CMV, HbA1c, anticardiolipin, lupus anticoagulant, anti-B2 glycoprotein-1 antibodies

Placental abruption or infarction - anticardiolipin, lupus anticoagulant, anti-B2 glycoprotein-1 antibodies

9. Induction of labour

Induction of labour is often required following a fetal death.

- There is little high-level evidence regarding optimal prostaglandin regimens
- Written consent must be obtained prior to the commencement of treatment. Consent should include Induction of Labour (IOL) +/- Manual Removal of Placenta (MROP)

Pharmaceutical methods of induction of labour

Second Trimester	Third Trimester
<u>Miscarriage/FDIU 14-28 weeks</u>	<u>FDIU > 28 weeks</u>
• Mifepristone 200mg orally (witnessed) – for priming • Followed 36 hrs later by: • Misoprostol 400mcg • Sub lingually 3 hourly • OR vaginally 4 hourly (max 5 doses) Previous caesarean: Half dose of Misoprostol In each case the dosage regimen should be discussed with the obstetric consultant and modified to the woman's particular medical circumstance.	• Mifepristone 200mg orally (witnessed) – for priming • Followed by 36 hrs later by vaginal prostaglandin (gel or pessary) • OR if Misoprostol is used the dose suggested is 400mcg vaginally 4 hourly *Misoprostol is not recommended > 28 weeks for the scarred uterus and should only be used at the discretion of the Consultant on call* Scarred Uterus • Mifepristone alone 200mg 8 hourly for up to 3 days can be considered in the IOL of FDIU in a woman with a scarred uterus • Once favourable an ARM can be done, followed by Oxytocin infusion as for VBAC

10. Intrapartum care

- Intrapartum care must be in line with any other relevant guidelines to ensure the most appropriate care is provided, including staffing considerations (experienced or support provided to those with less experience)
- Commence the Perinatal Loss Clinical Care Plan AKMR73.2
- Follow the flowchart appropriate for gestational age
- Adequate analgesia is particularly important when requested
- Active management of the third stage is recommended at all gestations
- Support requests to normalise the birth experience (e.g., partner cutting the umbilical cord)
- Handle the baby with care to prevent damage e.g., in case of skin slippage
- Continuity of care giver is best practice

- Ensure family members have private waiting areas (i.e., separate from other birthing families)

11. Investigations following birth

- Complete a clinical examination of baby and document according to the PSANZ Clinical Examination of Baby Checklist
- If NOT consenting to post-mortem – perform surface microbiological swabs (ear and throat) for microbiological culture, use two separate swabs and wet swabs prior to smearing
- If NOT consenting to post-mortem - clinical photographs of the baby are required. Photographs to be taken as outlined in [EMHS Clinical Photography and Videography Policy](#). Refer to Appendix 7
- Visual examination of placenta - record appearance of placenta, noting any obvious abnormalities of placenta, cord, or membranes
- If NOT consenting for post-mortem - Perform surface MC&S swabs – refer to Appendix 8 and send placenta for histopathology immediately
- Blood sample collection to be performed if there is a suspected chromosomal or metabolic disorder
 - Sampled from the cord by the midwife or cardiac puncture by the pathologist for investigations and blood group
- If consenting to a postmortem examination; Perinatal pathology will:
 - Collect blood samples for chromosomal analysis
 - Perform a baby gram at KEMH (x-ray)
 - Perform histopathology of the placenta (to be sent with baby to KEMH)

If parents do not agree to post-mortem, placental histopathology is highly recommended, in which it should be sent for histopathology at AHS immediately.

12. Postnatal care

- Expectation for length of stay inpatient is 24 hours after a vaginal birth and 48 hours after a caesarean section, an increase in length of stay will need to be discussed with the Midwifery Unit Manager or After-Hours Hospital Nurse Manager.
- Advise on lactation suppression and breast comfort. Women who have experienced a loss may be prescribed Cabergoline to suppress lactation. Information on lactation suppression can be found [here](#).
- Parents can be offered and encouraged to spend time with their baby. The use of a CuddleCot™ will allow the baby to stay with the parents while they are an inpatient. Refer to [Set-Up Manuals - CuddleCot](#) for manufacturer instructions.
- There is no evidence to support how long a baby should remain with the parents or how long they should be actively cooled after death. The emphasis is on the individual needs and wishes of the family. Care should be entirely family focused.
- Parents should be sensitively informed of the expected physical changes to their baby following death (i.e. colour changes, skin integrity and leakage of body fluids, odour). They should be reassured that these changes are expected and may be minimised by the correct use of the CuddleCot™.
- Educate families on the risk of deterioration, especially if they are intending to have a post-mortem examination. Encourage them to have some extended periods of time in the mortuary fridge or body hold fridge (for example over night when sleeping) and then have baby back onto the CuddleCot™ for some time during the day. Respect the families wishes if they do not wish to be separated from their baby.

- The body hold fridge in labour ward can be used to store the baby in periods between using the CuddleCot™. Ensure the body hold register is completed every time the baby is placed into the fridge or removed.
- Once the family has left and the baby is ready to be collected (by KEMH pathology or the funeral director) the baby must be transferred to the hospital mortuary (no babies can be left in the body hold fridge for collection by KEMH perinatal pathology or funeral directors). The AHNM should be notified of this transfer. Please follow the [Death of a Patient Procedure](#) in regards to how to transport the deceased baby through the hospital and admitting the baby into the mortuary.
- Ensure required paperwork is taken to the mortuary with the baby and left in the mortuary for KEMH perinatal pathology or funeral directors to collect.
- Once baby is admitted to the mortuary the midwife caring for the mother must call either the funeral directors or KEMH perinatal pathology to arrange collection of the neonate.
- Mementos should be created routinely for all perinatal deaths. Refer to Appendix 5 for creating mementos
- Discuss contraception if appropriate
- Ensure the family has written information on available support services
- Provide information on Centrelink Family Allowance Forms – Claim for Bereavement Payments for gestations >20 weeks

13. Care of the baby

The literature about contact with the baby is not certain. Most families will want to see, hold and spend time with their baby. Offer all families the opportunity to see and hold their baby. If families choose not to, they should be regularly re-offered the opportunity, however not coerced. Respect for cultures and compassionate sensitivity is required.

- Parents appreciate it when staff treat their baby with respect, such as calling the baby by name
- Mementoes should be created routinely for all perinatal deaths and offered to the family
- Every family must make some arrangements for the body of the baby, such as burial or cremation, depending on the circumstances and gestation.

14. Post-mortem (Autopsy)

A postmortem (or autopsy) is an examination of a body after death by a doctor, usually a pathologist. Postmortem examinations are performed to:

- Provide information about the cause of death
- Provide information about other possible inherited conditions and improve clinical care
- A postmortem should be offered to all parents following a stillbirth
- A brochure for parents can be located [here](#)
- Information for staff regarding how to discuss autopsy and what questions you may be asked can be found [here](#)
- All autopsy examinations require written consent following informed discussion
 - NCC Form 1 Pathwest "Consent for Postmortem Examination Non-Coronial"
 - NCC Form 2a should be completed for all miscarriages and stillbirths, and consenting to a post-mortem (regardless of gestational age) **or**,
 - NCC Form 2b should be completed if the baby was born alive and consenting to a post-mortem (regardless of gestational age)
- Discussion should include:
 - Options of exam: full, limited or external only

- The possibility that a cause may not be found
- Appearance of the baby following autopsy
- An estimated 6-8 weeks for a final report to be developed
- Follow up for results (referral to specialist obstetrician clinic 8-10 weeks postnatal)

In W.A. all postmortem examinations on newborns are carried out at KEMH. From February 2004, the transport and collection of all non-coronial perinatal deaths is co-ordinated from the Perinatal Pathology department at KEHM.

Contact 9340 2730 to arrange pickup for autopsy examinations.

This is a Mon-Fri 0730-1600 service. Leave a detailed message if after hours.

Note: If the death is to be investigated by the Coroner - leave all tubes in situ; curl up the catheters and tape to the baby, do not bath the baby.

Refer to Appendix 4 in the case of a Coroners Inquiry

The following should accompany the baby for autopsy

- Consent for post-mortem examination with detailed clinical history & copies of blood & USS results
- Placenta (fresh not in formalin)

Copies of:

- The Medical Certificate of Cause of Stillbirth or Neonatal Death certificate
- USS reports and Prenatal karyotyping results if available
- Consent for Cremation if <28 weeks

14.1 Alternative investigations when autopsy declined

A limited autopsy examination may yield useful information in situations where the parents decline full autopsy. Where parents decline full autopsy:

- Confirm that parents understand important information may be missed
- Offer parents options for:
 - External examination by a perinatal/paediatric pathologist, clinical geneticist, or paediatrician
 - Full body x-ray (baby gram KEMH)
 - Ultrasound scan
 - Clinical photographs
- Ensure request forms for pathology, histology or external examination clearly indicate the extent of consent

15. Taking baby home

Some parents may wish to take their baby home for periods of time. There is a requirement for a letter confirming the baby was stillborn (in case of official query e.g., during transport).

Request the GP obstetrician or Specialist Obstetrician to write on hospital letter head paper.

Before taking the baby home, the following must be considered.

- The effect of local climate on the body (i.e., temperature and humidity) advice about keeping the baby in a cool room and wrapped ice blocks kept on the cot mattress
- Completion of AKMR30.0 Release of human tissue and explanted device consent form

- Providing the Medical Certificate of Cause of stillbirth or Neonatal Death if care being transferred to the Funeral Director
- Legal requirements regarding birth registration, burial/cremation
- Arrangements for return to hospital or funeral home

16. Funeral arrangements

- It is a requirement for the parents to make arrangements for any stillbirth, miscarriage or neonatal death. Options include burial or cremation for a stillborn baby (refer to definition of stillbirth)
- Provide information regarding options for funeral/cremation arrangements (e.g., local funeral directors)
- If in-utero fetal death less than 20 weeks, the parents have the choice of taking the body home for appropriate burial. If so, completion of release forms is required
- Alternatively, the body can be cremated at KEMH for gestations <28 weeks. The ashes can then either be scattered in the rose garden, collected by the family or returned to the Midwifery manager at Armadale hospital to be collected by the family
- Provide information on opportunities to mourn the baby (e.g., hospital memorial services, remembrance services)

17. Follow up

- Upon discharge the woman will receive continued postnatal care at home through the AHS Visiting Midwife Service (VMS) up to day 10 postnatal. Ensure VMS paperwork is completed as per [AHS Discharge Maternity Guideline](#) .
- The midwife must complete a Special Child Health Nurse Referral prior to discharge home.
- An appointment is to be made with the Specialist Obstetrician involved in the care of the woman and her family at 8-10 weeks postnatally. The appointment is arranged for this period, as the investigations will be completed, and a medical practitioner can discuss any medical or pathology reports.
- The woman should see her GP in a few weeks for a general health check, a comprehensive summary should be sent to the GP.
- Refer to Appendix 6 for support services and organisations available to the family
- Consider the requirement for referral to relevant health care professionals and support groups prior to discharge – particularly for counselling/psychological support services (e.g., genetic counsellor, social worker, Child Health Services, pastoral care worker)
- Arrange follow up appointment(s) for the purposes of recurrence risk counselling and discussion of investigation results – first appointment 8-10 weeks postnatal
- Communicate and forward a comprehensive summary to the woman's General Practitioner, Paediatrician, and other relevant care providers

18. • Legislative context

The following documents are required to give effect to this policy:

- *Birth, Deaths, and Marriages Registration Act 1998*
- *Coroners Act 1996*
- *Health Act 1911*
- Health Department of Western Australia (HDWA) [Review of Death Policy MP 0098/18](#)
- HDWA [Release of Human Tissue and Expanted Medical Devices Policy MP 0129](#)

19. Supporting information

The following documents support the implementation of this policy:

- East Metropolitan Health Service [Consent Policy EMHS: 5](#)
- [Death of a Patient AKG-PRO-0388-14](#)
- [Mortuary Patients Procedure AKG-PRO-269-17](#)
- [Misoprostol and Mifepristone – Use of in Gynaecology and Obstetrics Guideline OBS-GUI-0266-15](#)
- [Midwifery Home Visiting Home Policy OBS-POL-0265-18](#)
- HDWA [Notification of perinatal and infant deaths](#)
- [Statewide Obstetric Support Unit: Perinatal Loss eLearning Package](#)

20. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 2 – Action 2.3 Healthcare rights and informed consent
- The health service organisation has a charter of rights that is:
 - a. Consistent with the Australian Charter of Healthcare Rights⁵³
 - b. Easily accessible for patients, carers, families and consumers
- Standard 5 – Action 5.15 Comprehensive care at the end of life
- The health service organisation has processes to identify patients who are at the end of life that are consistent with the National Consensus Statement: Essential elements for safe and high-quality end-of-life care²²⁰

21. References

1. RCOG Greentop Guideline No 55: Late Intrauterine Fetal Death and Stillbirth. October 2024 [Care of late intrauterine fetal death and stillbirth](#)
2. ACOG practice bulletin No. 102: Management of stillbirth. Obstetrics & Gynaecology. 2020; 135(3):748-761.
3. Welfare, A. G. A. I. o. H. a. (2025). *Australia's mothers and babies*. Australia Retrieved from <https://www.aihw.gov.au/reports/mothers-babies/australias-mothers-babies/contents/stillbirths-neonatal-deaths/stillbirths-and-neonatal-deaths-in-australia>
4. Queensland Clinical Guidelines. Stillbirth care. Maternity and Neonatal Clinical Guideline. 2018.
5. King Edward Memorial Hospital Clinical Guidelines. Perinatal Loss: Unexpected (miscarriage and stillbirth). Obstetric Clinical Guideline. 2024.
6. Page J, Silver, RM. Evaluation of stillbirth. Current Opinion in Obstetrics and Gynaecology. 2018;30(2).
7. Aminu M, Bar-Zeev S, van den Broek N. Cause of and factors associated with stillbirth: a systematic review of classification systems. Acta Obstet Gynecol Scand. 2017 May;96(5):519-528. doi: 10.1111/aogs.13126. PMID: 28295150; PMCID: PMC5413831.
8. Nijkamp J, Sebire, NJ, Bouman, k, Korteweg, FJ, Erwich, JJHM, Gordjin, SJ, . Perinatal death investigations: What is is current practice? Seminars in Fetal and Neonatal Medicine. 2017;22:167 - 175.
9. Page J, Silver, RM. Evaluation of stillbirth. Current Opinion in Obstetrics and Gynaecology. 2018;30(2).
10. FIGO's updated recommendations for misoprostol used alone in gynaecology and obstetrics. 2017
11. RANZCOG Miscarriage, Recurrent Miscarriage and Ectopic Pregnancy (C-Gyn 38) Clinical Guideline. March 2025.

22. Glossary

The following definitions are relevant to this policy.

Term	Definition
Stillbirth	A fetal death prior to birth of a baby of 20 or more completed weeks of gestation or of 400 grams or more birthweight.
Neonatal death	The death of a live born baby of 20 or more completed weeks of gestation or of <i>400 grams or more birthweight within 28 days of birth</i> .
Perinatal death	Stillbirth or neonatal death of a baby of 20 or more completed weeks of gestation or of 400 grams or more birthweight.
Intrapartum death	Fetal death occurring during labour and/or birth.

23. Document contact

Enquiries relating to this guideline may be directed to:

Title: Clinical Midwife & Perinatal Loss Co-ordinator
Department: Obstetrics and Gynaecology Unit
Email: ahs.perinatallossmidwife@health.wa.gov.au

24. Review

This guideline will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V4	14 10 2025	14 10 2028	Addition of Coroners Inquiry information and information re cuddle cot. Duplicated information removed from appendices.

25. Authorisation

This guideline has been authorised and issued by the Director Clinical Services.

Authorised by	Tania Strickland – Director Clinical Services
Authorisation date	10 10 2025
Published date	14 10 2025

Appendix 1: Reporting Aid

Gestation at birth	Weight	Definition	Register birth with Registrar of births, deaths and marriages?	Death certificate required?	Perinatal data collection reporting required?
<20 weeks	< 400g	Miscarriage or fetal death before 20 weeks	No	No	No
<20 weeks	< 400g	Born alive (gasps/heartbeat)	Yes	Yes	Yes
<20 weeks	≥ 400g	Stillbirth	Yes	Yes	Yes
≥20 weeks	≥ 400g	Stillbirth	Yes	Yes	Yes
≥ 20 weeks	< 400g	Stillbirth	Yes	Yes	Yes
≥ 20 weeks and proven fetal death in-utero at < 20 weeks (proven by ultrasound)	Any weight	Fetal death before 20 weeks	Optional*	Optional*	Yes

Appendix 2: Documentation for Fetus <20 weeks gestation

Document	Documentation for fetus less than 20 weeks
BDM 100	BDM 100 Birth registration form – for all the births greater than 20 weeks, regardless of outcome, and for live born babies less than 20 weeks gestation . Parents complete this and return the form to the Registrar Generals office
Consent for pathology	NCC Form 1 – Pathwest “Consent for Postmortem – Non-Coronial” here NCC Form 2a – Pathwest “Clinical information for miscarriages, fetal death or stillbirth”
Bereavement	Bereavement payment – not eligible for <20 weeks' gestation
Registration	Registration – if live born or stillborn > 20 weeks or ≥400g must be registered. If the loss took place before 20 weeks and the baby weighed less than 400g a ‘Recognition of Early Pregnancy Loss’ Certificate is available online from the WA Registry of Births, Death & Marriages – found here
Postmortem	Postmortem – pathology examination is offered at KEMH on the baby less than 20 weeks gestation
Placenta Histopathology	For histopathology of placenta – KEMH will accept AHS forms.
Photos/memory box	Photos/Memory box – photos & prints, if possible, for memory box
Cremation or funeral	The baby can be taken home for appropriate disposal. Completion of Authorisation and ‘Release of Human Tissue and Explanted Medical Device Consent Form’. WA Health Patient Information Sheet and Consent Form – Authorisation and Release of Human Tissue Form is required or KEMH Cremation. Ensure NCC form 3 is completed. The family can collect ashes or have them interred at KEMH Rose garden. Ashes can also be sent to a GP or maternity unit manager to be collected by the family. Options to be provided to the patient
The Statutory Law	The Statutory Law states that gestation is assessed according to the duration of pregnancy not intra-uterine fetal life
Midwives Notification	Midwives Notification Stork – No stork to complete for a pregnancy less than 20 weeks gestation

Appendix 3: Documentation for Fetus 20-28 weeks' gestation

Document	Documentation for fetus 20-28 weeks
BDM 100	BDM 100 Birth registration form – for all the births greater than 20 weeks, regardless of outcome, and for live born babies less than 20 weeks gestation. Parents complete this and return the form to the Registrar Generals office
Form 7	Form 7 Certificate of Medical Attendant. Cremation Act 1929, for any live born baby or stillborn
Postmortem	Consent for post-mortem examination. Used in conjunction with the information for parent's pamphlet. This clearly explains all procedures and options regarding post-mortem examination. A midwife may sign as a witness. NCC Form 1 – Pathwest "Consent for Postmortem – Non-Coronial" here
Placenta Histopathology	For histopathology of placenta – KEMH will accept AHS forms.
BDM 201	BDM 201 Medical Certificate of cause of stillbirth or neonatal death – completed by the attending doctor and goes with the baby to KEMH. Pathology returns it to the Registrar Generals office as notification of death (for Statutory Requirements for Executive Director Public Health).
Cremation or Funeral Director	KEMH Cremation is available free of charge until 28 weeks. Ensure NCC form 3 is completed. The family can collect ashes or have them interred at KEMH Rose garden. Ashes can also be sent to a GP or maternity unit manager to be collected by the family. Options to be provided to the patient Or an outside funeral director can be arranged
The Statutory Law	The Statutory Law states that gestation is assessed according to the duration of pregnancy not intra-uterine fetal life
CIMS	CIMS form is required to be completed for any stillbirths caused by omission of care or sentinel events
STORK	Midwives' notification. Labour and birth summary information is reported to WA Health by Midwives as Notification of Case Attended (NOCA) for all births. This may be via STORK database where in use.
Bereavement Payment	Bereavement payment – Claim for Bereavement Payment form (from Family Assistance Office, ph.: 13 61 50). If eligible for bereavement payment the parents have two choices 1. The one-off baby bonus or 2. The 12-week maternity leave entitlement They are only eligible for this if they would otherwise be paid on maternity leave. The booklet is in the perinatal loss box file.
Photos & Memory Box	Photos and Memory Box
Death in Hospital	Notification of death shall occur for any child of more than 20 weeks gestation is stillborn or any child under the age of one year shall die from any cause whatsoever, the fact shall be reported forthwith to the Executive Director, Public Health. WA Health Review of Death Policy MP 0098/18 Email copy of "Medical Certificate of Cause of Stillbirth or Neonatal Death" Email: edphwa@health.wa.gov.au Fax: 9222 2322
Coronial Enquiry	FOR CORONIAL INQUIRY http://www.kemh.health.wa.gov.au/development/manuals/O&G_guidelines/sectiona/8/a8.3.3.pdf

Appendix 4: Documentation for Fetus >28 weeks gestation

Document	Documentation for fetus >28 weeks
BDM 100	BDM 100 Birth registration form – for all the births greater than 20 weeks, regardless of outcome, and for live born babies less than 20 weeks gestation. Parents complete this and return the form to the Registrar Generals office
Form 7	Form 7 Certificate of Medical Attendant. Cremation Act 1929, for any live born baby or stillborn
Postmortem	Consent for post-mortem examination. Used in conjunction with the information for parent's pamphlet. This clearly explains all procedures and options regarding post-mortem examination. A midwife may sign as a witness. NCC Form 1 – Pathwest "Consent for Postmortem – Non-Coronial" here
Placenta Histopathology	For histopathology of placenta – KEMH will accept AHS forms.
BDM 201	BDM 201 Medical Certificate of cause of stillbirth or neonatal death – completed by the attending doctor and goes with the baby to KEMH. Pathology returns it to the Registrar Generals office as notification of death (for Statutory Requirements for Executive Director Public Health).
Cremation or funeral	Stillborn babies greater than 28 weeks gestation must have funeral arrangements made through an external funeral director. This is managed by Pastoral Care Services in conjunction with the family.
The Statutory Law	The Statutory Law states that gestation is assessed according to the duration of pregnancy not intra-uterine fetal life
Midwives Notification	Midwives Notification Stork – No stork to complete for a pregnancy less than 20 weeks gestation
CIMS	CIMS form is required to be completed for any stillbirths caused by omission of care or sentinel events
STORK	Midwives' notification. Labour and birth summary information is reported to WA Health by Midwives as Notification of Case Attended (NOCA) for all births. This may be via STORK database where in use.
Bereavement Payment	Bereavement payment – Claim for Bereavement Payment form (from Family Assistance Office, ph.: 13 61 50). If eligible for bereavement payment the parents have two choices 3. The one-off baby bonus or 4. The 12-week maternity leave entitlement They are only eligible for this if they would otherwise be paid on maternity leave. The booklet is in the perinatal loss box file.
Photos & Memory Box	Photos and Memory Box – completion of grief pack booklet If the patient declines printed photos, then they can be given the option of taking them home in a <u>sealed</u> envelope or left out. Photos not to be stored in patient file –
Death in Hospital	Notification of death shall occur for any child of more than 20 weeks gestation is stillborn or any child under the age of one year shall die from any cause whatsoever, the fact shall be reported forthwith to the Executive Director, Public Health. WA Health Review of Death Policy MP 0098/18 Email copy of "Medical Certificate of Cause of Stillbirth or Neonatal Death" Email: edphwa@health.wa.gov.au Fax: 9222 2322
Funeral	All perinatal deaths over 28 weeks or any neonatal deaths are managed by a private funeral director of the parents' choice
Coronial Inquiry	FOR CORONIAL INQUIRY Review of Death Guideline

AKMR6 Death in Hospital – if any questioned answered as “yes” in Section 2 of the AKMR6 then the death is **REPORTABLE** to the Coroner, the process is outlined in the **AKMR6**
Consent to postmortem and cremation are **not applicable** in a Coroners Inquiry
BDM 201 Medical Certificate of cause of stillbirth or neonatal death is **not applicable**

The family should still have all the appropriate care and referrals as above.

The family should be advised to not book a funeral date as the time needed to conduct the inquiry can vary, they can be given information on funeral directors for after the inquiry is complete.

Appendix 5: Creating Mementos

Respect	• Treat the deceased baby with the same respect as a live baby (e.g., handle baby with care, use name if one was given) • Support parents to feel in control of the care of their baby • Respect the wishes/preferences of parents when offering care • Respect cultural and religious beliefs/practices/rituals
Memory creation	Offer time with baby – inform parents they may hold, undress and bath baby if desired (complete all swabs and tests on baby before bathing) • Offer for both parents, family and siblings (if parents think it's appropriate) to spend time with the baby • Provide a 'Grief pack' with brochures • Memento booklet • Grief & support handouts • Parental keep sake photographs. Call 'Heartfelt' 1800 583 768 • Offer options to include extended family (e.g., photographs of family groups, relatives/siblings to hold baby, video conferencing if available) • Offer option to take baby home if feasible • Facilitate religious/cultural rituals and services • Facilitate memento creation/gathering following parental consent (e.g., identification tags, hand and footprints, digital photographs, cot cards, hair collection) •
Funeral Arrangements	• Discuss naming/blessing/baptism services and other religious rituals • Refer to perinatal pathology handbook for additional information regarding autopsy examinations and cremation services at KEMH. Discuss funeral plans including considerations of • Cremation options • Private funeral director arrangements (cremation or burial)

Appendix 6: Organisations & Support Services

Organisation	Contact Details
Red Nose	Advocate for and fund research into stillbirth and other areas of sudden and unexpected child death. Extend bereavement support and counselling to families who have experienced stillbirth or the sudden and unexpected death of a child, regardless of the cause. Web: Red Nose Australia Bereavement support phone: 1300 308 307 (24 hours)
SANDS WA	Provides support, information, education and advocacy for parents and their families who have suffered the loss of a baby through miscarriage, stillbirth, neonatal death and other reproductive losses. Offers support via telephone and support group meetings SANDS WA: 1300 072 637 https://www.sands.org.au/
Heartfelt (formerly Australian Community of Child Photographers)	Professional photographers dedicated to providing photographic memories to families that have experienced stillbirths, premature and ill infants and children in the Neonatal Intensive Care Units of their local hospitals, as well as children with serious and terminal illnesses. All services are provided free of charge. Phone: 1800 583 768
Pregnancy and Infant Loss Australia	A support program for bereaved families who have experienced loss through miscarriage, stillbirth, genetic inducement of labour or neonatal death. Bereavement support phone 1800 824 240 (No local contact)
Lifeline	Provide telephone crisis support to anyone needing emotional support Phone: 13 11 14
Angelhands Inc	Offers support and strength to people who affected or bereaved by violence https://angelhands.org.au
Australian Centre for Grief and Bereavement	This service provides details of free bereavement counselling services and links to websites. Free call: 1800 642 066
Australian Funeral Directors' Association	Web: Australian Funeral Directors Association Phone: (08) 9355 3441
Beyond Blue	This service assists those with depression and anxiety problems Web: Beyond Blue Free call: 1300 224 636
Compassionate Friends	An international family support organisation working to bring hope to families who have lost a child Web: Compassionate Friends
Crisis Care 24-hour support	Phone: (08) 9223 1111 or STD Free call 1800 199 008 or TTY (08) 9325 1232 Accessed through the telephone interpreter service.
dNet	Provides information and support to people with depression, including families and friends Web: dNet
Family Helpline	Free telephone counselling and information service for families with relationship difficulties Phone: (08) 9223 1100 or free call 1800 643 000
Mensline	The service provides counselling services especially for men

	Web: MensLine Australia Free call: 1300 789 978
Metropolitan Cemeteries Board	For information about burial, cremation and services Web: Metropolitan Cemeteries Board Phone: (08) 9383 5200
Solace Association	Offers support for people who have lost their partner. Services also extended to affected children Web: Solance Australia Free call: 1300 308 307 or Phone: (08) 9359 3892

Appendix 7: Clinical Photographs of Baby Guide

Instructions on taking clinical photographs	- As per EMHS Clinical Photography and Videography policy - High quality medical photographs are preferred; however, Polaroid pictures are better than no pictures at all. Ideally digital photographs should be taken which will allow the clinician to check each photo after it is taken. - These photographs should be taken in addition to bereavement photographs. - Refer to laminated pictorial instructions in perinatal loss folder
Consent	- Parental consent is necessary prior to taking clinical photographs. - Document the consent in the medical notes. - Due to their clinical nature, it is strongly recommended that the parents are not offered copies, but specific bereavement photographs are taken instead. - These only need to be done if there is no postmortem being done. - Ensure the baby is always handled with respect and sensitivity.
Background	- Plain drapes (blue is the best background – other backgrounds may create glare or alter skin tone). - Use of a blue theatre would suffice.
Scale	- Place a paper tape measure next to the baby (plastic ruler will create glare) - Ensure zero is aligned at the base of the foot or crown of the head - Use sticky tape to ensure the tape is straight; and - Measure should be on the bottom of the frame or the left
Identification	- Write the baby's UR number on the paper tape measure for identification. - Don't write any other identifying information in case the photographs are ever mislaid.
Setting	Photographs should be taken in a private area away from the parents.
Technique	- The photographs should be taken from directly above the baby. - Consequently, it is best to place the baby on the floor, to get sufficient height above the baby.
Magnification	- Use a 50mm lens/magnification for the whole-body photographs and maintain a consistent distance. - Use a 100mm lens/magnification (except for digital) for the facial photographs, filling the whole frame.
Baby	The baby should be naked for all the photographs.
Position	- AP view – whole body including limbs - PA view – whole body including limbs - Lateral view of the body - Lateral views of the face - Frontal view of the face - Photographs of any abnormalities
Storing of Clinical Photographs	- Photos are to be placed in a sealed envelope - Patient Identification label to be placed on front of the envelope - Mark the envelope with "Sensitive Images" and how many photos are contained within the envelope. - Send photos to medical records where they will be logged with a "Clinical Photograph header sheet" and archived.

Appendix 7: Example of Clinical Photographs of Baby

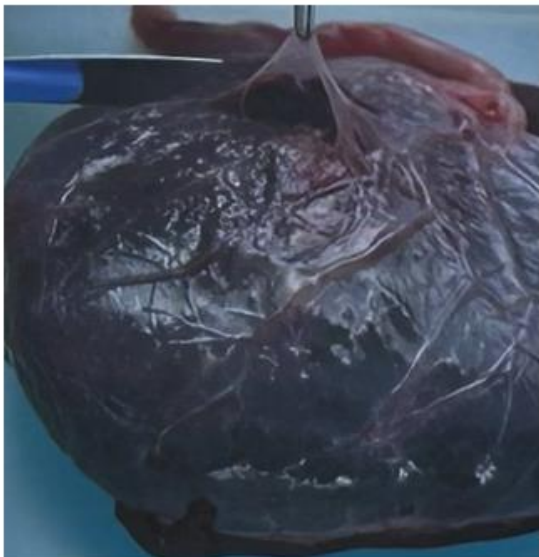
AP View – Whole body frontal including limbs  • Tape measure to the left • Palms facing up	PA View – Whole body back including limbs  • Keep the baby in this position for the minimum time possible. • Tape measure to the left • Palms facing down
Lateral view of the body  To stabilise: • Pull underneath arm forwards • Legs in 'running position' • Top arm and leg will fall forward which will aid stability • Keep the tape measure to the left	Frontal view of the face  • Ensure tape measure is in the frame.
Right lateral views of the face 	Left lateral views of the face  • Keep tape measure to the left of the frame to aid easy identification of the side being viewed.

Appendix 8: How to swab a Placenta



HOW TO SWAB A PLACENTA FOR MICROBIOLOGY

1. Lift amnion to make a cut

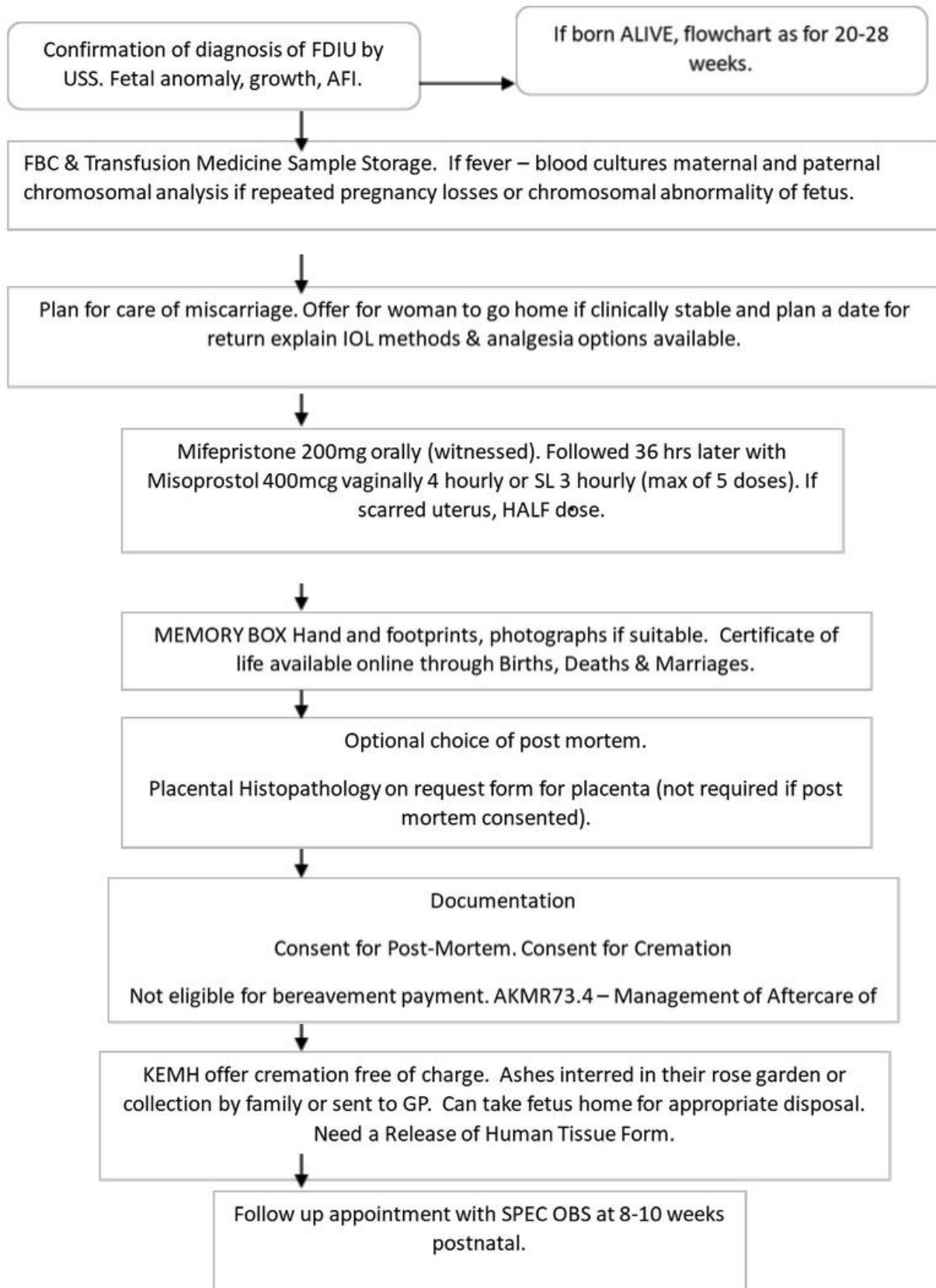


2. Incise and lift amnion off surface to expose chorion

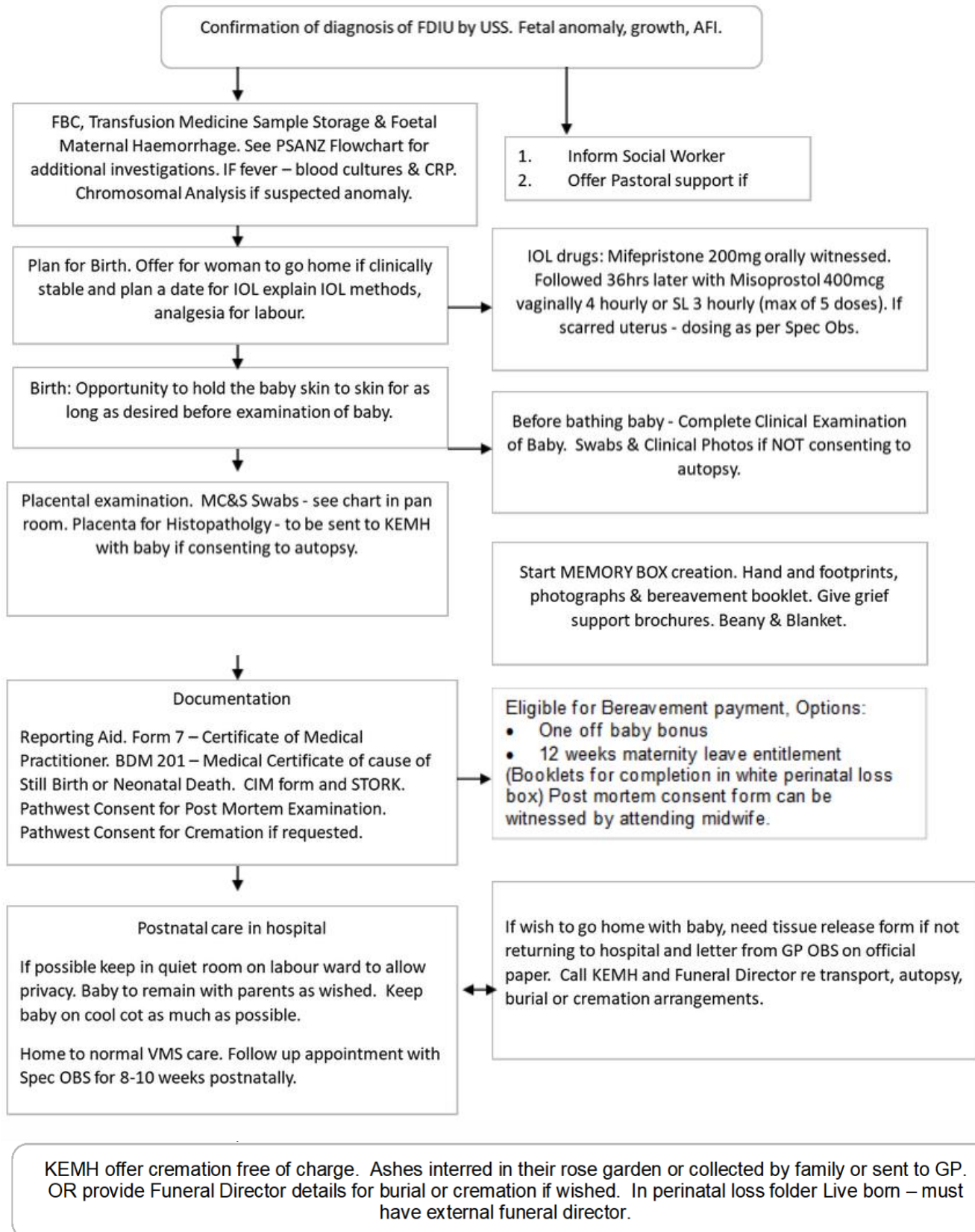


3. Swab under amnion well away from incision site.

Appendix 9: Pregnancy loss 14-20 weeks flowchart



Appendix 10: Perinatal loss 20-28 weeks flowchart



Appendix 11: Perinatal loss > 28 weeks flowchart

